

Derbyshire Shared Care Pathology Guidelines

Gout in Adults

Purpose of Guideline

This guideline covers the management of Gout in Primary Care and interface with Secondary Care. It does not include tumour lysis syndrome, which is managed within Oncology.

Measuring urate during an acute attack of possible gout

SUA falls during an acute attack of gout, resulting in 30-40% of patients having a normal level at this time. It is therefore recommended to use clinical judgement and consider aspirating joint fluid to make the diagnosis at this stage. SUA should be measured at least 2 weeks after the acute flare has settled

How should an asymptomatic patient with a high SUA be managed?

Only 1 in 20 patients with a high SUA will develop gout so a high urate does not need treating per se. However, the following factors should be considered and addressed where appropriate:

- Lifestyle: intake of alcohol, red meat, seafood, sugar sweetened soft drinks
- Medication: diuretics (thiazide, loop), ciclosporin, cytotoxic drugs, tacrolimus
- High urate is seen in association with the 'metabolic syndrome' so consider monitoring for dyslipidaemia, type 2 diabetes and renal disease, and management of cardiovascular risk factors

How should an acute attack of gout be managed?

Consider and investigate alternative diagnoses, especially septic arthritis. If septic arthritis is considered, DO NOT start antibiotics, but refer URGENTLY to orthopaedics/ED for joint aspiration for gram stain and culture of fluid and blood.

If alternative diagnoses ruled out see NICE

[Visual summary - management of gout \(nice.org.uk\)](https://www.nice.org.uk/guidance/TA50) for advice on management of gout flares

DO NOT STOP allopurinol (or other urate lowering therapies) during an attack

- Adjunctive management includes:
 - Other analgesia if necessary
 - Rest and elevate the limb.
 - Keep the joint exposed and in a cool environment.
 - Consider the use of an ice pack or bed-cage.

- Return if symptoms get worse, or if there is no improvement after 1-2 days.
- Follow up at 4–6 weeks
 - Assess lifestyle factors
 - Assess gout activity / diagnosis
 - Check blood pressure
 - If the patient taking diuretics for hypertension, review whether an alternative agent is possible. Losartan has mild uricosuric effects
 - Check SUA, renal function and HbA1c; consider lipids

Long Term management of Gout

NICE Visual summary

[Visual summary - long-term management of gout with ULTs \(nice.org.uk\)](https://www.nice.org.uk/guidance/NG195/visual-summary)

Contains information on

Who urate lowering therapy (ULT) should be offered to

See <https://cks.nice.org.uk/topics/gout/> and local medicines guidelines for up to date pharmaceutical management of gout noting that adjustments may be required in renal impairment.

• What are the aims in gout management?

In a patient with gout, the optimum SUA level is < 360 µmol/L*

- Aim for a target SUA of < 360 µmol/L (6 mg/dL), but consider a lower target serum urate level below 300 µmol/L (5 mg/dL) for people with tophaceous gout, chronic gouty arthritis or with recurrent flares despite achieving a target of 360 µmol/L
- Reduce the risk of recurrent attacks of gout if not outweighed by the risks of treatment
- Minimise the risk of developing complications of gout, such as joint damage, renal stones, and urate nephropathy

When to consider referral to secondary care?

Consider referring to the appropriate specialty in the following situations:

- If the diagnosis is uncertain
- Response to treatment has not been adequate or treatment is not tolerated
- Treatment is contraindicated
- Gout in a young person (<25 yrs) or premenopausal female, when a genetic enzyme defect needs to be considered
- Refractory acute attack
- Complications, e.g. stones, nephropathy or troublesome tophi
- Patient has previously had an organ transplant

Contacts

Derby/

Duty Biochemist	01332 789383 (8am to 7pm, Mon – Fri)
Rheumatology Advice (9-5, Monday-Friday)	Via RDH switchboard, 01332 340131
Renal Advice (9-5, Monday-Friday)	Via RDH switchboard, 01332 340131

Chesterfield

Duty Biochemist	01246 512212 (9am to 5pm, Mon – Fri)
Rheumatology Advice (9-5, Monday-Friday)	Via CRH switchboard, 01246 277271
Renal Advice (9-5, Monday-Friday)	Via CRH switchboard, 01246 277271

Burton

Duty biochemist	01283 (8.30 - 4pm, (diverts to Derby outside of these hours))
Rheumatology Advice (9-5, Monday-Friday)	Via QHB switchboard, 01283
Renal Advice (9-5, Monday-Friday)	Via QHB switchboard, 01283

References

Doherty M *et al.* Efficacy and cost-effectiveness of nurse-led care involving education and engagement of patients and a treat-to-target urate-lowering strategy versus usual care for gout: a randomised controlled trial. *Lancet*. 2018;392:1403-1412

Stamp LK *et al.* A randomised controlled trial of the efficacy and safety of allopurinol dose escalation to achieve target serum urate in people with gout. *Ann Rheum Dis*. 2017;76(9):1522-1528

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Hui M *et al.* The British Society for Rheumatology guideline for the management of gout. *Rheumatology*. 2017;56(7):e1–e20

FitzGerald JD *et al.* 2020 American College of Rheumatology Guideline for the Management of Gout. *Arthritis Care Res (Hoboken)*. 2020;72(6):744-760

NICE NG219 2022 Gout: Diagnosis and Management

Patient information

Written information and patient support can be found via the UK Gout Society. See www.ukgoutsociety.org

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June 2012	1	Dr L Badcock, Dr M Livingston	New Document
June 2014	2	Dr L Badcock, Dr P Blackwell, Ms H Seddon	Routine review
Feb 20217	3	Dr L Badcock, Dr P Blackwell, Ms H Seddon	Routine review
Dec 2017	4	Dr M Hui, Dr L Badcock, Rheumatology team, Renal team, S Dhadli (Medicines Management)	Routine Review
May 2021	5	Dr M Hui, Dr L Badcock, UHDB Rheumatology team, UHDB Renal team, Dr K Fairburn, S Dhadli (Medicines Management), Dr P Blackwell, Mrs H Seddon	Routine review
Nov 2023	6	Dr B Kasem, Dr M Hui, UHDB Rheumatology team, A Thai (Medicines Management), Dr P Blackwell, Mrs H Seddon	Routine review
Approval	Date	Name(s)	Designation
Secondary Care UHDB	07.0426	Dr R Laxminanarayan	Consultant Rheumatologist
Primary Care	24.03.26	Dr P Blackwell	GP
Laboratory (Derbyshire Pathology)	24.04.26	Ms S Knowles	Consultant Clinical Scientist